CHOTANAGPUR PATHOLOGY & MOLECULAR LAB
Histopathology | Immunohistochemistry | Tumour Markers
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HISTOPATHOLOGY REPORT

Name:	**AMIT KUMAR MUNDA**	Path No.:	**CPL/H/26/4418**
Age / Sex:	22 / M	Sample received:	04-Jul-2026
Referred by:	Dr. Prashant Tirkey, Subarnarekha Cancer Hospital	Date of report:	**10-Jul-2026**
Specimen:	Right high inguinal orchidectomy (operated 03-Jul-2026)	Clinical:	Testicular mass; AFP and beta-hCG raised

Gross

Testis with epididymis and spermatic cord (9 cm). Testis 6 x 4.5 x 4 cm; cut surface shows a variegated grey-white to yellow tumour 4.7 cm with haemorrhage, necrosis and small cysts, replacing most of the testis and abutting the tunica albuginea. Cord cut end inked.

Microscopy

Mixed germ cell tumour: embryonal carcinoma (about 60 %), yolk sac tumour (about 30 %, reticular and microcystic patterns) and mature teratoma (about 10 %). No seminoma or choriocarcinoma component. Germ cell neoplasia in situ in the adjacent tubules.

Synoptic summary

Parameter	Finding
Tumour size	4.7 cm
Rete testis	Stromal invasion present
Tunica albuginea / vaginalis	Albuginea breached focally; tunica vaginalis free
Lymphovascular invasion	**Present**
Epididymis	Free
Spermatic cord resection margin	Free of tumour
pT (AJCC 8th ed.)	**pT2**

Final diagnosis

Right testis, high inguinal orchidectomy: MIXED GERM CELL TUMOUR (embryonal carcinoma 60 %, yolk sac tumour 30 %, teratoma 10 %) with lymphovascular invasion.

Immunohistochemistry (OCT3/4, SALL4, glypican-3, CD30) supports the components described; slides available for review.

Dr. Anupama Ekka
M.D. (Pathology)
Reg. No. JMC-3087
Consultant Pathologist

*** End of Report ***

SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY